

PILOT Surgical Audit and Standards Network

Record ID

Medical Record Number:

Date of birth of the patient

Date on admission

Age of the patient at time of surgery (Months/Years)

Gender of the patient

- ☐ Male
☐ Female
☐ Ambiguous Genitalia
☐ Other

Patient's weight

- ☐ Done
☐ Not Done

Patient's weight (kg)

Patient's Height

- ☐ Done
☐ Not Done

Patient's Height (in meters)

BMI (kg/m²)

HB level (g/dL) immediate before surgery

Pre-operative Diagnosis

Date of surgery

Primary indication for surgery:

- ☐ Infective
☐ Malignancy
☐ Non-communicable disease
☐ Injury or Trauma
☐ Caesarean section
☐ Inflammatory conditions e.g. IBD, Diverticular Disease
☐ None of the above
☐ Other

If Other, Please specify

If trauma, what specifically?

- ☐ Work-related
- ☐ Road-traffic
- ☐ Fall
- ☐ Natural disaster
- ☐ Violence
- ☐ Other

Age group of the patient

- ☐ Paediatric patient
- ☐ Adult patient

Type of surgery (Adults)

- ☐ Orthopaedic and Trauma
- ☐ Gynaecology and obstetrics
- ☐ Oncologic/cancer surgery
- ☐ Gastrointestinal and hepatobiliary (upper and lower gastrointestinal)
- ☐ Urology and kidney
- ☐ Head and neck
- ☐ Cardiothoracic and vascular (cardiac, vascular, thoracic lung, and thoracic gut)
- ☐ Neurosurgery (procedures involving the brain and cervical spine)
- ☐ Otorhinolaryngology
- ☐ Plastic surgery
- ☐ Other unclassified surgeries

If other, please mention:

Type of Surgery performed (ADULT)

- ☐ Hernia inguinal - all types
- ☐ Appendicectomy (all types)
- ☐ Exploratory laparotomy (no other procedure)
- ☐ Wound - debridement / haematoma / delayed closure
- ☐ Excision of skin lesion, skin tumours skin closure, superficial abscess drainage
- ☐ Drainage of superficial abscess
- ☐ Breast lump - excision
- ☐ Caesarean section
- ☐ Small bowel (resection and primary anastomosis, Duodenum - perforated DU closure)
- ☐ Thyroid - subtotal thyroidectomy or lobectomy
- ☐ Colostomy - formation of end / loop colostomy / ileostomy / double barrel colostomy
- ☐ Biliary - cholecystectomy
- ☐ Mastectomy - simple + node biopsy/sample/clearance
- ☐ Split skin graft
- ☐ Trauma - laparotomy/laparoscopy
- ☐ Colectomy - sigmoid - complete operation
- ☐ Amputation
- ☐ Excision of haemorrhoids
- ☐ Feeding jejunostomy or gastrostomy
- ☐ Hemicolectomy and anast colon / rectum
- ☐ Colostomy closure
- ☐ Excision of skin lesion, flap/graft
- ☐ Splenectomy (any cause)
- ☐ Fracture long bone (upper/lower) - ORIF

If the type of procedure is not listed, please add here:

Types of Surgery (Paediatrics)

- ☐ Gastro-intestinal
- ☐ Paediatric Gynaecological
- ☐ Hepatobiliary
- ☐ Oncological
- ☐ Otorhinolaryngology
- ☐ Plastic Surgery
- ☐ Splenectomy
- ☐ Thoracic
- ☐ Paediatric Trauma and Orthopaedics
- ☐ Urological
- ☐ Neurosurgery
- ☐ Neonatal Surgery
- ☐ Congenital surgery
- ☐ Other

If other, please mention

Type of Surgery Performed (PAEDIATRICS)

- ☐ Hernia inguinal - paediatric
- ☐ Colostomy - formation of end / loop colostomy / ileostomy / double barrel colostomy
- ☐ Repair of umbilical hernia
- ☐ Bilateral orchidopexy
- ☐ Insertion of VP shunt
- ☐ Exploratory laparotomy (no other procedure)
- ☐ Circumcision - paediatric
- ☐ Hypospadias repair
- ☐ Closure of colostomy
- ☐ Posterior sagittal anorectoplasty
- ☐ Appendicectomy (all types)
- ☐ small bowel resection and primary anastomosis
- ☐ Open reduction of intussusception
- ☐ Heminephrectomy
- ☐ Pyloromyotomy for paediatric pyloric stenosis
- ☐ Anoplasty
- ☐ Wound - debridement / haematoma / delayed closure
- ☐ Trache-oesophageal fistula & oesophageal atresia repair
- ☐ Trans-anal pull-through
- ☐ Excision of skin lesion, skin tumours skin closure, superficial abscess drainage
- ☐ Anal dilatation/calibration
- ☐ Swenson Pull-through
- ☐ Cystoscopy
- ☐ Excision of retrorectal tumour / sacrococcygeal teratoma
- ☐ Surgical Closure of Gastroschisis
- ☐ Central line insertion
- ☐ Refashioning of colostomy
- ☐ Closure of Exomphalos
- ☐ Closure of ileostomy
- ☐ Correction of malrotation
- ☐ Repair of small bowel perforation
- ☐ Duodenoduodenostomy
- ☐ Excision/ligation of patent processus vaginalis
- ☐ Oesophageal dilatation
- ☐ Chest drain - insertion
- ☐ Circumcision
- ☐ Creation of temporary ileostomy
- ☐ Biopsy soft tissue
- ☐ Skin Graft
- ☐ Cleft and Palate Surgery

If the type of pediatric surgery is not listed, please add here:

Did the patient have any of these preoperative chronic comorbid conditions (Check all that apply)

- ☐ Coronary artery disease
- ☐ Congestive heart failure
- ☐ Diabetes mellitus
- ☐ Metastatic cancer
- ☐ Hypertension
- ☐ Stroke
- ☐ Chronic obstructive pulmonary disease
- ☐ HIV
- ☐ Chronic renal disease
- ☐ Asthma
- ☐ Blood related conditions (e.g. Sickle cell Anaemia, Haemophilia)
- ☐ Cirrhosis
- ☐ Malnutrition
- ☐ None of the above
- ☐ Other specify

If others, please mention:

Urgency of Operation

- ☐ ELECTIVE (intervention planned or booked in advance of routine admission to hospital. Timing to suit patient, hospital and staff)
- ☐ URGENT (intervention for acute onset or clinical deterioration of potentially life-threatening conditions, for those conditions that may threaten the survival of limb or organ, for fixation of many fractures and for relief of pain or other distressing symptoms. Normally within hours of decision to operate)
- ☐ EMERGENCY (Immediate life, limb or organ-saving intervention - resuscitation simultaneous with intervention. Normally within minutes of decision to operate. A) Life-saving B) Other e.g., limb or organ saving)

ASA Physical Status Score

- ☐ ASA I - Normal healthy individual
- ☐ ASA II - Mild systemic disease that does not limit activity
- ☐ ASA III - Severe systemic disease that limits activity but is not incapacitating
- ☐ ASA IV - Incapacitating systemic disease which is constantly life-threatening
- ☐ ASA V - Moribund, not expected to survive 24 hours with or without surgery

Was Prophylactic Antibiotics given pre-operative?

- ☐ YES
- ☐ NO
- ☐ Not Indicated

If YES, what was the Prophylactic Antibiotic given?

- ☐ Ampicillin/Sulbactam
- ☐ Cefazolin
- ☐ Cefoxitin
- ☐ Cefepime
- ☐ Clindamycin
- ☐ Gentamicin
- ☐ Metronidazole
- ☐ Ceftriaxone
- ☐ Amikacin
- ☐ Amoxicillin+clavulanic acid
- ☐ Piperacillin+sulbactam
- ☐ Azithromycin
- ☐ Vancomycin
- ☐ Cefuroxime
- ☐ Cefradine
- ☐ Ciprofloxacin
- ☐ Other

If other, please specify:

Route of Prophylactic Antibiotic Administration

- ☐ Intravenous (IV)
- ☐ Oral
- ☐ Intramuscular
- ☐ Intradermal
- ☐ Other

Please approximate the timing of Prophylactic Antibiotics administration

- ☐ 30-60 minutes before Surgical Incision
- ☐ 1-2 hours before Surgical Incision
- ☐ 2-3 hours before Surgical Incision
- ☐ 3-4 hours before Surgical Incision
- ☐ 4-5 hours before Surgical Incision
- ☐ Within one hour (60 minutes) after Surgical Incision
- ☐ Not Indicated

Was Pharmacological Venous Thromboembolism Prophylaxis prescribed?

- ☐ YES
- ☐ NO

If Yes, VTE Prophylaxis was provided:

- ☐ Within 24 h prior to surgery
- ☐ 24 h after surgery
- ☐ Not Indicated

Preoperative fasting

- ☐ Adequate preoperative fasting: e.g. clear fluids up to 2 h prior to surgery, solids up to 6 h prior to surgery
- ☐ Prolonged fasting with no clear fluids
- ☐ No fasting

What was the Level of the most senior surgeon who participated in procedure?

- ☐ Superspecialist (paediatric surgery, gastrointestinal surgery, surgical oncologist)
- ☐ Specialist (General surgeon)
- ☐ Medical doctor/registrar
- ☐ Mmed resident 1
- ☐ Mmed resident 2
- ☐ Mmed resident 3
- ☐ MCS 1
- ☐ MCS 2
- ☐ FCS 1
- ☐ FCS 2
- ☐ FCS 3
- ☐ FCS 4

What was the level of the most senior Anaesthesia Provider for this procedure in the OR?

- ☐ Specialist anaesthetist
- ☐ Anaesthesia trainee (fellow/resident)
- ☐ Anaesthesiologists (Non-specialist physician anaesthetist)
- ☐ Non-physician anaesthetist
- ☐ Surgeon
- ☐ FCS trainee
- ☐ MCS trainee
- ☐ Other

Was the surgical field preparation done according to local standards?

- ☐ YES
- ☐ NO

Anaesthesia type used (select all that apply)

- ☐ Local anaesthesia
- ☐ Regional anaesthesia
- ☐ General anaesthesia
- ☐ Monitored anaesthesia care or sedation
- ☐ WALANT
- ☐ No anaesthesia

Estimated blood loss (in mls)

Length of surgery (In minutes)

Were minimal invasive procedures e.g. laparoscopy used for this operation?

- ☐ YES
- ☐ NO
- ☐ YES but converted to Open Surgery

What was the class of the wound?

- ☐ CLASS I (Clean surgical wounds show no signs of inflammation and do not involve the respiratory, gastrointestinal or genitourinary tracts)
- ☐ CLASS II (Clean-contaminated wounds are clean wounds with a higher risk of infection such as those involving the gastrointestinal, respiratory or genitourinary tracts, as long as the surgery is uncomplicated. Any wound opened to remove pins or wires, chest procedures, ear surgeries or gynecologic procedures)
- ☐ CLASS III (Contaminated wounds - created when an outside object e.g a bullet, knife blade or other pointy object comes in contact with the wound. Or caused by large amounts of spillage from the GI tract into the wound or any highly inflamed or infected tissue around a surgical wound)
- ☐ CLASS IV (Dirty-infected surgical wounds include those with a foreign object lodged in the wound (such as a bullet or other debris), traumatic wounds from a dirty source where the treatment was delayed, infected surgical wounds or any wound that has been exposed to pus or fecal matter)

Post-operative diagnosis

Intraoperative monitoring and checklist

	YES	NO	Not Applicable
Was there a continuous intraoperative monitoring using pulse oximeter?	☐	☐	☐
Was a continuous intraoperative monitoring using ECG?	☐	☐	☐
Was there continuous use of carbon dioxide monitoring using capnography?	☐	☐	☐
Was the WHO Surgical Safety Checklist completed?	☐	☐	☐
Was Intraoperative Antibiotics provided?	☐	☐	☐
Was the patient given Intra-Operative Blood Transfusion?	☐	☐	☐

If blood transfusion was given, how many units?

Was Enhanced Recovery After Surgery (ERAS) protocol for this patient care?

- ☐ YES
- ☐ NO
- ☐ Not Applicable

What was the patient's disposition?

- ☐ General ward
- ☐ HDU
- ☐ ICU
- ☐ Death on the table
- ☐ Discharge Home

Did the patient require re-operation or return to theatre within 30 days of index operation?

- ☐ YES
- ☐ NO
- ☐ Not Applicable

If YES please mention the reason

Did the patient receive pain medication within 24 hours after Surgery?

- ☐ YES
- ☐ NO
- ☐ Not Applicable

What pain medication/s did the patient receive within first 24 hours after surgery?

- ☐ Opioids (e.g Tramadol, Pethidine)
- ☐ NSAIDs (e.g Ibuprofen, Diclofenac) and/or acetaminophen
- ☐ Gabapentin or pregabalin
- ☐ I.V. Ketamine
- ☐ I.V. Lidocaine
- ☐ Acetaminophen
- ☐ Other

If Other, Please specify:

Post-operative complications (Claiven Dindo Classification)

- ☐ GRADE I - Any deviation from the normal postoperative course without the need for pharmacological treatment or surgical, endoscopic and radiological interventions. Allowed therapeutic regimens are drugs as antiemetics, antipyretics, analgesics, diuretics and electrolytes and physiotherapy, including wound infections opened at the bedside)
- ☐ GRADE II - Requiring pharmacological treatment with drugs other than such allowed for grade I complications. it includes Blood transfusions and total parenteral nutrition.
- ☐ GRADE III - Requiring surgical, endoscopic or radiological intervention
- ☐ GRADE IIIa - Intervention not under general anesthesia
- ☐ GRADE IIIb - Intervention under general anesthesia
- ☐ GRADE IV - Life-threatening complication (including CNS complications) requiring IC/ICU-management
- ☐ GRADE IVa - Single organ dysfunction (including dialysis)
- ☐ GRADE IVb - Multiorgan dysfunction
- ☐ GRADE V - Death of a patient

Complication Severity Grading	☐ MILD COMPLICATION (results in only temporary harm and does not usually require specific clinical treatment) ☐ MODERATE COMPLICATION (a more serious, usually requires clinical treatment but does not usually result in permanent harm or functional limitation) ☐ MAJOR COMPLICATION (is one that results in significant prolongation of hospital stay and/or permanent functional limitation or death, and almost always requires clinical treatment)
Did the patient develop post operative Surgical Site Infection?	☐ YES ☐ NO
If yes which type?	☐ Superficial surgical site infection ☐ Deep surgical site infection ☐ Organ space surgical site infection
What was the outcome of the patient at 30 days?	☐ Alive and discharged ☐ Patient died
Date of discharge (dd/mm/yyyy)	_____
On discharge from hospital, complete the following checklist on Morbidity developed during any 24 h period of postoperative hospital admission (multiple answers possible) - for adapted Postoperative Morbidity Survey	☐ Pulmonary - Has the patient developed a new requirement for oxygen or respiratory support during postoperative admission? ☐ Did the patient receive antibiotics and/or had a temperature of > 38 degrees C during postoperative admission? ☐ Renal-Presence of oliguria 200 mL/hr or pharmacological therapy), atrial or ventricular arrhythmias, cardiogenic pulmonary oedema, thrombotic event (requiring anticoagulation) ☐ New focal neurological deficit, confusion, delirium, or coma ☐ Hematological - Requirement for any of the following: packed erythrocytes, platelets, fresh-frozen plasma, or cryoprecipitate ☐ Wound - Wound dehiscence requiring surgical exploration or drainage of pus from the operation wound with or without isolation of organisms ☐ Pain - New postoperative pain significant enough to require parenteral opioids or regional analgesia ☐ No Complication developed
Did the patient develop any disability?	☐ YES ☐ NO
If YES, please specify	_____
Cause of death	☐ Myocardial injury after noncardiac surgery ☐ Major bleeding (Hemorrhage) ☐ Sepsis ☐ Cardiac failure ☐ Multiorgan failure ☐ Stroke ☐ Other causes

If Other, please specify: